

Post-Acute Care Follow-Up Call Tool

CMT: CMT-0016 (SR)

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Introduction

A follow-up phone call after a hospitalization allows the provider to evaluate the patient after hospital discharge to ensure that the patient has received and understands self-management instructions.(1) This call also gives the provider an opportunity to identify and prevent potential adverse events.(2) Providers can potentially intervene to prevent adverse events during the follow-up call by giving additional patient education, initiating post-acute service (eg, home healthcare), or coordinating an appointment with an outpatient provider.(3)(4) Patients and caregivers benefit from the post-acute discharge phone call because it provides them with a mechanism to ask questions or get clarification without having to navigate what may be perceived as a complex system. Additional benefits include reducing patient anxiety and complaints.(5)

Hospital providers should tell the patient during discharge education that the patient will be called within 2 to 4 days after hospital discharge. Clinical staff from the patient's discharge unit are the best candidates to call the patient. This allows for continuity of care and a familiarity with the patient. Outcomes from the calls can then be tracked to learn from the issues identified on the calls and to provide feedback to staff regarding preventable readmission issues.(3)(6)

Post-Acute Care Follow-Up Call Questionnaire

- **Follow-up appointments** questions to ask the patient include:
 - Do you have an appointment with your primary care provider (PCP) for follow-up?(2)(7)
 - Name of provider: _____
 - Date and time of appointment: _____
 - Do you have an appointment with a specialist physician (eg, surgeon, neurologist) for follow-up?
 - 1. Name of provider: _____ ; Date and time of appointment: _____
 - 2. Name of provider: _____ ; Date and time of appointment: _____
 - 3. Name of provider: _____ ; Date and time of appointment: _____
 - 4. Name of provider: _____ ; Date and time of appointment: _____
 - 5. Name of provider: _____ ; Date and time of appointment: _____
 - 6. Name of provider: _____ ; Date and time of appointment: _____
 - Do you have a way to get to and from provider appointments?
- **Health status** questions include:
 - Did you get a discharge summary? If so:
 - Did you (or your caregiver) read it?
 - Do you understand it? (See Teach Back Tool  SR for information and instruction in performing assessment of patient's understanding of self-management activities.)
 - Do you have any questions about it?
 - Do you have any new or worsening symptoms? Specific questions based on disease or condition status include:
 - Do you have a lingering or new fever?
 - Does your bandage look clean, dry, and intact?
 - Are you eating and drinking OK?
 - Do you have any new pain?
 - Have you gained weight?
 - Do you have regular bowel movements?

- Do you have problems going to the bathroom?
- Do you know what to do if your symptoms worsen?
 - What symptoms should you call the doctor or surgeon with?
 - What symptoms should you get help right away with?
 - Do you know how to contact the doctor that treated you during the hospital stay?
- Do you have any questions about your medicines?
 - Do you have a list of all the medicines that were prescribed?
 - Are all the medicines you were prescribed in your home?
 - Are you taking all the prescribed medicines? If not, why?
 - Did not pick them up
 - Does not think they are needed
 - Cannot afford them
 - Did not know new medicine was prescribed
 - Does not believe in taking medicine
 - Does not like the side effects
 - Other: _____
 - Do you know what your medicine does?
- Do you have any questions about how you should take care of yourself? Specific questions based on disease or condition status may include:
 - Do you know what you can and cannot eat?
 - Do you know how active you can be?
 - Do you know how to care for your wound?
 - Do you know how to care for your cast or immobilizer?
 - Do you know how and when to check your weight?
 - Do you know what to do if you have swelling?
 - Do you know what to do if you become short of breath?
 - Do you know how to check your blood pressure?
 - Do you know how to check your blood sugar?
 - Do you know how to check your heart rate?
- Do you have any problems with your activities of daily living or your instrumental activities of daily living? Specific questions may include:
 - Do you have any problems with bathing or showering?
 - Do you have trouble eating or feeding yourself?
 - Do you have trouble getting out of bed or going to the toilet?
 - Are you able to move around as expected?
 - Are you able to perform shopping tasks and plan and prepare light meals?
 - Are you able to perform housekeeping tasks and do laundry?
 - Do you have any trouble managing your medications?
- Do you have equipment needs? If yes:
 - Do you have the equipment your doctor ordered?
 - Can you use the equipment properly and safely?
- Other: _____
- **Interventions provided**[A](1)
 - Patient/caregiver education provided
 - Patient appointment(s) scheduled
 - Provider contacted
 - Home care services arranged
 - Social services contacted
 - Transportation arranged
 - Pharmacy contacted
 - Long-term care referral made
 - Another follow-up call scheduled
 - Medical equipment coordinated
 - Insurance company contacted (eg, for case management or condition management)
 - Other: _____

Resources

- Additional information, resources, and tools include:
 - After Hospital Care Plan from the Project Re-Engineered Discharge (RED) Toolkit, Boston Medical Center at www.bu.edu/fammed/projectred
 - How to Conduct a Post-discharge Follow-up Phone Call from the Project Re-Engineered Discharge (RED) Toolkit, Boston Medical Center at www.bu.edu/fammed/projectred

- Health Care Professionals, Transitions of Care Checklist from the National Transitions of Care Coalition (NTOCC) at www.ntocc.org
- How-to Guide: Improving Transitions from the Institute for Healthcare Improvement at www.ihl.org

References

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5. Re-Engineered Discharge (RED) Toolkit. AHRQ Publication No. 12(13)-0084 [Internet] Agency for Healthcare Quality and Research. 2020 Feb Accessed at: <https://www.ahrq.gov/>. [created 2013; accessed 2023 Oct 09] [Context Link 1]
6. Designing and delivering whole-person transitional care: the hospital guide to reducing Medicaid readmissions. (Prepared by Collaborative Healthcare Strategies, Inc., and John Snow Inc., under Contract No. HHSA290201000034I). AHRQ Publication No. 16-0047-EF. [Internet] Agency for Healthcare Research and Quality. 2016 Sep Accessed at: <https://www.ahrq.gov/>. [accessed 2023 Oct 06] [Context Link 1]
7. Nall RW, Herndon B, Mramba LK, Vogel-Anderson K, Hagen MG. An interprofessional primary care based transition of care clinic to reduce hospital readmission. American Journal of Medicine 2020;133(6):e260-e268. DOI: 10.1016/j.amjmed.2019.10.040. [Context Link 1] View abstract...

Footnotes

[A] Interventions performed should be done with the consent of the patient (or appropriate caregiver) unless there is a legal reporting issue identified (eg, suspected abuse). Provider orders should be obtained for medical services that may be needed, prior to service initiation (eg, prescriptions for medications, skilled care services order for home health).(1) [A in Context Link 1]

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